What your cholesterol *is whispering.*

A heart and brain protection guide from a night hospitalist

For anyone who has been told their cholesterol is "a little high," anyone already on treatment, and anyone who wants to understand their numbers before they become a hospital story.

"But doctor, my cholesterol was only a little high."

I hear some version of this on night shift, often from someone who has just had a heart attack or a stroke. Years ago, a lab report flagged their LDL. It didn't hurt. They felt fine, life was busy, and the number quietly kept doing its work on the walls of their arteries. By the time we meet, it's 3 a.m. and we're treating the result. This guide is about meeting that number much earlier, calmly, while you still have every option.

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THE ONE LESSON

Your cholesterol is an early signal about your heart and brain. You can act on it long before anything happens.

PART 1

Why cholesterol *matters to me*

I see cholesterol from two sides. At work, I rarely meet it on its own. I meet what it helped build over many years: the heart attack, the stroke, the leg that lost its blood flow. And at home, I'm watching my own. Here is the pattern behind those nights.

01 LDL builds plaque, slowly

Extra LDL particles slip into artery walls and settle there. Over years they form plaque. It narrows the artery, and it can crack open and trigger a clot.

02 It's the years that add up

Plaque reflects how high your LDL has been and for how long. That's why the newest guideline encourages lowering it earlier in life, not only after something happens.

03 The heart, the brain and the legs

The same plaque process sits behind most heart attacks, many strokes, and narrowed arteries in the legs. One number touches all three.

04 It travels with other conditions

High blood pressure, diabetes, kidney disease and smoking each add to the risk. Together they multiply it. Cholesterol is one thread you can actually pull.

05 Feeling fine tells you nothing

High cholesterol has no symptoms. The only way to know it is a blood test, and the only way to know your plan is working is to check again.

06 Much of it is preventable

The American College of Cardiology and American Heart Association estimate that 80% or more of cardiovascular disease can be prevented. That's the hopeful part of this story.

07 Medicine is support, not a failure

Some people reach their goals with lifestyle changes. Many need medicine as well, often because of genes they inherited. Needing help isn't a sign you did something wrong. It's a sign your body needs more support.

Why this one is personal

My own LDL is higher than I'd like. Not dramatically, but enough that I noticed, and enough that I'm paying attention. So I'm doing what this guide asks of you: reading labels, adding fiber, moving more, and rechecking my numbers. I know what "a little high" can become, because I meet it at 3 a.m. I'm not writing this from the finish line. I'm on the same journey, and I'm choosing to listen early.

PART 2

Know *your numbers*

A standard cholesterol panel gives you four numbers. Two more tests are worth knowing about. Write your latest results in the blanks so they're in one place.

LDL-C

THE MAIN TARGET

Often called "bad" cholesterol. The particles that build plaque. Lower is better, especially if your risk is higher.

Mine: _____ mg/dL Date: _____

HDL-C

NOT A SHIELD

Often called "good" cholesterol. A high HDL doesn't cancel out a high LDL, so don't let it reassure you on its own.

Mine: _____ mg/dL Date: _____

Triglycerides

FATS IN THE BLOOD

Often rise with excess weight, diabetes, alcohol and refined carbohydrates. Very high levels can also inflame the pancreas.

Mine: _____ mg/dL Date: _____

Non-HDL-C

THE FULLER PICTURE

Total cholesterol minus HDL. It counts every plaque-building particle, and it's reliable whether or not you fasted.

Mine: _____ mg/dL Date: _____

Lp(a)

CHECK ONCE

Lipoprotein(a) is mostly inherited and isn't part of a standard panel. The guideline now says every adult should have it measured at least once.

Mine: _____ Checked? ☐ Yes ☐ Not yet

ApoB

SOMETIMES ADDED

Counts the plaque-building particles directly. Useful for some people, such as those with diabetes or high triglycerides, especially once LDL is at goal.

Mine: _____ Date: _____

IF YOUR SITUATION IS…	LDL-C GOAL	NON-HDL-C GOAL
Preventing a first heart attack or stroke, at low, borderline or intermediate risk	below 100	below 130
Preventing a first heart attack or stroke, at high risk	below 70	below 100
You've already had a heart attack, stroke or similar event, and you're at very high risk	below 55	below 85

Goals in mg/dL, from the 2026 ACC/AHA guideline. Your clinician sets your own goal based on your full picture. Bring this page and ask which row is yours.

PART 3

What your PREVENT score *means for you*

PREVENT is the American Heart Association's newer risk calculator, and it's now recommended for adults ages 30 to 79 who haven't had a heart attack or stroke. It estimates your chance of a heart attack or stroke over the next 10 years, and over the next 30.

YOUR 10-YEAR RISK CATEGORY

under 3%	3 to 5%	5 to 10%	10% +
LOW	BORDERLINE	INTERMEDIATE	HIGH
Healthy habits are the plan. Keep checking.	A conversation about whether medicine adds benefit.	Medicine is often recommended alongside lifestyle.	Stronger treatment and a lower LDL goal.

The higher the risk, the stronger the recommendation and the lower the goal on page 3.

What goes into it

- Age and sex
- Total cholesterol and HDL
- Blood pressure, and whether you take medicine for it
- Diabetes and smoking
- **Kidney function**, which the older calculator ignored
- Whether you already take a statin, and sometimes your A1c or a urine protein test

PREVENT doesn't use race. It was built to be more accurate than the calculator it replaced.

Why the 30-year number matters

If you're in your 30s or 40s, your 10-year risk will look small almost no matter what, because time hasn't caught up yet. The 30-year estimate shows where your current numbers are leading. A low 10-year risk with a high 30-year risk is a whisper worth listening to.

When the score isn't the whole story

Your clinician may also weigh "risk enhancers," such as a high Lp(a), family history of early heart disease, or early menopause. A coronary calcium (CAC) scan can help when the decision is uncertain.

Ask at your next visit

"What is my PREVENT 10-year and 30-year risk, and does it change my plan?" My 10-year: _____ % My 30-year: _____ %

PART 4

Before your *lab draw*

A little preparation keeps your results accurate, so your plan is built on the right numbers.

Do I need to fast?

For most people, **no**. A non-fasting cholesterol test is fine for screening and for most follow-up checks. LDL barely changes after a meal.

Your clinician may ask you to fast if:

- A non-fasting triglyceride result came back at 400 or higher
- You have a known triglyceride problem
- You have a strong family history of high cholesterol or early heart disease

If you're asked to fast, water is fine. Follow the instructions on your lab order.

Bring your supplement list

- [ ] **Red yeast rice.** Contains the same compound as a prescription statin.
- [ ] **Fish oil** or other omega-3s
- [ ] **Niacin** (vitamin B3)
- [ ] **Biotin**, often in hair, skin and nail products and B-complex vitamins
- [ ] Anything "natural" for the heart, plus herbal teas and powders

A note on biotin

Biotin doesn't usually change a standard cholesterol panel. But high doses can throw off other tests often drawn at the same visit, like thyroid tests, and the FDA has warned it can make a heart attack test (troponin) read falsely low. Tell your clinician and the lab if you take it, and ask whether to pause it before your next blood draw. Don't skip that conversation just because the cholesterol test itself is safe.

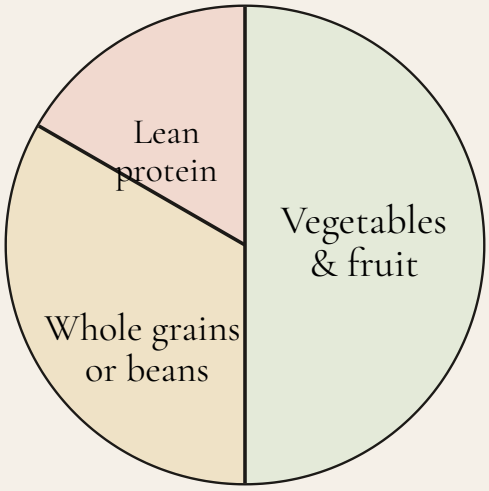
Your lab-day checklist

- [ ] Know whether this test is fasting or non-fasting
- [ ] Take your usual medicines unless told otherwise
- [ ] Drink water
- [ ] Bring your medicine and supplement list
- [ ] Ask whether Lp(a) is on the order if you've never had it
- [ ] Ask how and when you'll get your results

PART 5

Food that *moves the numbers*

The guideline points to Mediterranean and DASH-style eating. Think about what you can add before what you have to remove. Small swaps, repeated, are what show up in your next lab result.



Add more of these

- ☐ **Soluble fiber:** oats, barley, beans, lentils, apples, citrus. It helps carry cholesterol out of the body.
- ☐ **Nuts and seeds**, a small handful most days
- ☐ **Olive or canola oil** in place of butter, lard or coconut oil
- ☐ **Fish** a couple of times a week, plus poultry and plant proteins
- ☐ **Vegetables** at most meals, half the plate when you can

Swaps that lower LDL over time

~~Butter on toast~~
→ Avocado or a little olive oil

~~Sausage or bacon breakfast~~
→ Oatmeal with fruit and nuts

~~Fried chicken~~
→ Baked or grilled chicken

~~Fatty red or processed meat most days~~
→ Beans, fish or poultry most days

~~Pastries, cookies, chips~~
→ Fruit, nuts, roasted chickpeas

~~Sugary drinks~~
→ Water, unsweetened tea, sparkling water

A label clue

On the Nutrition Facts label, look at **Saturated Fat**. 5% Daily Value or less per serving is low, and 20% or more is high. Check for "partially hydrogenated oil" in the ingredients too, a source of trans fat. Sugars and refined carbs matter most for triglycerides.

If your triglycerides are high, a registered dietitian can make this plan fit your culture, your budget and your kitchen.

PART 6

Beyond *the plate*

Your arteries respond to your whole week, not only your meals. These habits work alongside food and medicine, not instead of them.

01 Move most days

Brisk walking counts. Work toward about 150 minutes a week, plus some strength work. Movement lowers triglycerides and helps your whole risk picture.

02 Stop smoking and vaping

Smoking damages artery walls so plaque settles more easily. Stopping is one of the biggest single changes you can make. Ask for help, because it works better with support.

03 Weight, if it's right for you

Losing even 5% of body weight can lower triglycerides and blood sugar. Ask your clinician what's realistic and safe for you.

04 Alcohol: less is better

Alcohol can raise triglycerides, sometimes sharply. If yours are high, ask whether cutting back or stopping makes sense.

05 Sleep and stress

Short sleep, untreated sleep apnea and constant stress all push blood pressure and blood sugar the wrong way. They're part of heart care too.

06 Manage the other numbers

Blood pressure, blood sugar and kidney health feed the same risk. Keeping them in range protects the same arteries.

Some causes aren't about habits

An underactive thyroid, kidney disease, some medicines and pregnancy can all raise cholesterol. So can genes. About 1 in 250 to 300 adults has familial hypercholesterolemia, an inherited condition with very high LDL from a young age. If your LDL is 190 or higher, or close relatives had heart attacks or strokes at a young age, ask about it. Family members may need testing too.

My family history

Heart attack, stroke or stent in a parent, brother or sister?
☐ Yes ☐ No ☐ Not sure

Who, and at what age? (Early means under 55 for men, under 65 for women.)

PART 7

Your cholesterol *after menopause*

Many women tell me, "I eat the same way I always have, and my cholesterol still went up." That's a real pattern, not a failure of willpower. The years around menopause change how the body handles cholesterol.

01 LDL tends to rise

The American Heart Association notes that LDL and ApoB rise during the menopause transition, more than aging alone would explain. Estrogen falls, and so does some of its protective effect.

02 HDL may protect less

Research suggests a higher HDL doesn't reliably mean better heart health after the transition. A "great HDL" is another reason not to ignore LDL.

03 The body changes shape

Fat shifts toward the belly and muscle mass declines. Both nudge triglycerides, blood sugar and blood pressure upward.

04 Timing matters

Earlier menopause, before age 45, or surgery that removes both ovaries, is linked to higher heart risk. Make sure your clinician knows when yours happened.

05 Symptoms can be clues

Frequent hot flashes and night sweats have been linked with worse heart-risk factors. They're worth mentioning at a heart-health visit, not only at a gynecology visit.

What helps in this season

- [ ] Recheck your panel, rather than relying on old "normal" results
- [ ] Strength training two or more days a week to protect muscle
- [ ] Fiber-rich, Mediterranean-style meals
- [ ] Ask for your PREVENT score and an Lp(a) if you've never had one

About hormone therapy

Hormone therapy is mainly used to treat menopause symptoms, not to lower cholesterol. Whether it's right for you depends on your age, how long it's been since menopause, and your own risks. Have that conversation with your clinician, and keep your cholesterol plan going either way.

PART 8

If you're on *medication*

Statins remain the foundation of treatment. When they aren't enough, or aren't tolerated, there are other options. Here's how to get the most from whatever you take.

01 Take it consistently

Cholesterol medicine protects you while you take it. Pick a time you'll remember each day, and use a pill box or phone reminder if it helps.

02 Better numbers mean it's working

When LDL drops, that's usually the medicine doing its job, not a sign you can stop. Numbers often climb back within weeks of stopping.

03 Muscle aches? Speak up

Tell your team instead of quietly stopping. Often a different statin, a lower dose or a different schedule works. Report severe muscle pain or dark urine right away.

04 There's more than one option

If a statin isn't enough or isn't tolerated, other medicines can be added, such as ezetimibe, bempedoic acid or PCSK9 inhibitors.

05 Check for interactions

Some antibiotics, antifungals, heart medicines and large amounts of grapefruit juice can interact with certain statins. Ask your pharmacist when anything new is added.

06 Pregnancy plans

Most cholesterol medicines are paused when trying to conceive, during pregnancy and while breastfeeding. Talk with your clinician before you stop.

Red yeast rice isn't a gentler swap

It contains monacolin K, the same compound as the prescription statin lovastatin. The amount varies from bottle to bottle, and supplements aren't checked the way prescription medicines are. Don't take it with a statin, and tell your clinician if you're using it.

My medicines

MEDICINE AND DOSE	WHEN I TAKE IT	STARTED	SIDE EFFECTS TO MENTION

PART 9

Your lab tracker, and *when to act*

Your numbers only tell a story when you can see them side by side. Here's the usual rhythm. Your clinician may adjust it for you.

How often to recheck

- **Adults, starting at age 19:** a cholesterol panel about every 5 years if all is well
- **After starting or changing medicine:** recheck in about 4 to 12 weeks
- **Once you're stable on treatment:** usually every 6 to 12 months
- **Lp(a):** once in adulthood is usually enough

Recheck sooner when

- Your medicine or dose changes
- You've made a big lifestyle change
- You're entering menopause
- You develop diabetes, kidney disease or another new condition

DATE	LDL	HDL	TRIGLYCERIDES	NON-HDL	ON MEDICINE?	WHAT CHANGED

WHEN IT CAN'T WAIT · CALL 911

Signs of a heart attack

Chest pain, pressure or squeezing. Pain in the arm, back, neck, jaw or stomach. Shortness of breath, a cold sweat, nausea or lightheadedness. Women are more likely to have the "other" symptoms, sometimes without obvious chest pain.

Signs of a stroke: BE FAST

B Balance	E Eyes	F Face drooping
A Arm weakness	S Speech	T Time to call

Call 911. Don't drive yourself, and don't wait to see if it passes. Every minute matters.

PART 10

Your *appointment page*

Bring this guide, your tracker and your medicine list. Start with the first question. The rest will help you leave with a clear plan.

1 "What is my LDL goal, and how close am I?"
Your goal depends on your risk. Write it on page 3.

2 "What is my PREVENT 10-year and 30-year risk?"
Ask what that score means for your plan.

3 "Have I ever had my Lp(a) checked?"
If not, ask whether it can be added to your next blood draw.

4 "Is my current plan enough, or do I need more support?"
That could mean lifestyle help, a medicine, or a change in dose.

5 "When should I recheck my labs, and should I fast?"
Write the date below so it doesn't slip.

6 "Could my menopause, medicines or family history be part of this?"
Bring your family history from page 7.

My goal: LDL below _____ Next lab date: _____ Notes:

Your cholesterol isn't a grade. *It's a whisper,* and you heard it early.

Every check, every swap, every dose you take on schedule is protecting your heart and your brain years from now. You don't have to do it perfectly. You only have to keep listening. I'm listening to mine too.

Dr. Ijay
INTERNAL MEDICINE PHYSICIAN · NIGHT HOSPITALIST · COGNITEVE_MD

@cogniteve_md

General education, not personal medical advice. This guide doesn't replace care from your own clinician, who knows your history. Don't start, stop or change a medicine without talking to them. In an emergency, call 911. Sources: 2026 ACC/AHA Guideline on the Management of Dyslipidemia; American Heart Association scientific statement on the menopause transition and cardiovascular risk (2020); U.S. FDA safety communication on biotin interference with lab tests; American Heart Association and American Stroke Association warning-sign guidance.